Cetuximab + cisplatin + fluorouracil

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 8 and 15 · cetuximab-only visit within the combination cycle

Active medicines: Cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Antihistamine plus corticosteroid	Individual prescribed agent, dose and route	At least one hour before cetuximab Mandatory infusion premedication. Reconcile any low-risk antiemetic need with this steroid; no automatic platinum prophylaxis.
Rescue antiemetic	As individually prescribed	If required Review persistent nausea; do not reuse the Day 1 platinum course.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00417 v7 · pNCCP 00417: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00417 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required